

Mosquito Bite - Female (Child)

Algorithm 1240 | Child (0-17 years) | Gender at birth: Female

Definition

- Qatar-localized UAT-only adult and pediatric pathway for mosquito bites, screening for anaphylaxis, infection and travel-related febrile disease; not approved for production.
- Child patient.
- Female.

Initial assessment

1. [LOCATION] Where are the bites?
2. [OPEN_TEXT] Any recent travel to an area with mosquito-borne disease risk?
3. [TEMPERATURE] Any fever?
4. [OPEN_TEXT] What is the exact age; is the patient pregnant; and are there bleeding, severe headache, confusion, vomiting, dehydration, widespread rash, immune suppression or safeguarding concerns?
5. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Anaphylaxis

Are throat or tongue swelling, breathing difficulty, faintness, confusion, or collapse the main concern?

Systemic allergic-reaction features require the Anaphylaxis pathway.

1. Disposition level 100

Are the lips, mouth, throat, or tongue suddenly swollen, is the person struggling to breathe, or has anyone lost consciousness or become floppy?

NHS.UK insect bites and stings guidance lists these as signs of a serious allergic reaction (anaphylaxis) requiring an immediate 999 call.

1. Disposition level 70

Is there fever, chills, or feeling generally unwell following recent travel to an area with mosquito-borne disease risk (e.g. malaria, dengue)?

Fever after travel-related mosquito exposure needs prompt evaluation to rule out mosquito-borne illness - a standard travel-medicine caution beyond the source page's own bite-specific criteria.

1. Disposition level 50

Is the skin around the bite hot, red, and painful, or leaking pus or fluid (signs of infection)?

NHS.UK guidance flags these infection signs as needing a pharmacist or primary-care review.

1. Disposition level 50

Are these typical itchy mosquito bites with none of the features above?

NHS.UK guidance describes minor bites without infection or allergy signs as manageable at home.

Advice

Emergency allergic reaction precautions

- Call Qatar 999 and do not drive. Use only the patient's own prescribed adrenaline auto-injector according to its plan and dispatcher instructions; keep the patient lying unless breathing is difficult. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: symptoms persist or recur; loss of consciousness.

Urgent travel-fever review

- Arrange prompt medical evaluation for fever following travel to a mosquito-borne disease risk area - mention the travel history clearly.
- Call Back If: fever worsens; new symptoms develop.

Routine bite follow-up

- Keep the area clean and book a routine review for signs of infection.
- Call Back If: increasing redness or pain.

Review for presumed mosquito bites

- Avoid scratching, clean the area gently and arrange clinician/pharmacist review before medication selection. Record travel precisely and escalate any fever or systemic symptom.
- Call Back If: infection, bleeding or widespread rash; fever, confusion, vomiting or dehydration.

First aid

- Call Qatar 999 and do not drive. Use only the patient's own prescribed adrenaline auto-injector according to its plan and dispatcher instructions; keep the patient lying unless breathing is difficult. Call Qatar 999 now, follow

the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Insect bites and stings", <https://www.nhs.uk/conditions/insect-bites-and-stings/> (page last reviewed 01 June 2023)

Provenance

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